

CONTINUED FROM PREVIOUS PAGE	1. PATIENT INITIALS (first, last) UNKNOWN	24b. MFR CONTROL NO. 2024A098631
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7 + 13 DESCRIBE REACTION(S) (including relevant tests/lab data) [Continuation]:

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Concomitant medication included amlodipine mesilate for hypertension (since 01-JAN-2003), levothyroxine sodium for hypothyroidism (since 01-JAN-2003), erythropoietin for a hypercholesterolemi (since 12-MAY-2023), atorvastatin calcium for calcium supplement (since 19-MAY-2023), boron, calcium citrate, colecalciferol, copper, magnesium hydroxide, manganese, silicon, zinc for anaemia (since 16-JUN-2023), pantoprazole sodium sesquihydrate for anemia (from 01-APR-2024 to 04-APR-2024), ondansetron for prophylaxis for acidity (from 23-APR-2024 to 25-APR-2024), potassium chloride for prophylaxis for nausea and vomiting (from 23-APR-2024 to 25-APR-2024), potassium chloride for treatment for hypokalemia (from 23-APR-2024 to 25-APR-2024) and ferrous ascorbate, folic acid for 3) oofer xt(ferrous ascorbate + folic acid).

Study therapy started on 19-MAY-2023 for breast cancer.

On 23-APR-2024, the patient experienced generalized weakness is a symptom of diarrhoea (preferred term: Diarrhoea). Subject came with complaints of generalized weakness, nausea, giddiness and loss of appetite on 23 Apr 2024. The subject had suffered from diarrhoea (3-4 episodes on 22 Apr 2024 and one episode on 23 Apr 2024). Subject completed Cycle 11 of AZD9833/Placebo 75 mg, Palbociclib 125 mg and Anastrozole/Placebo 1 mg. Further, subject was admitted for medical management and hence SAE was reported. During the hospital stay, she was treated conservatively, subject was administered antibiotics, antiemetics and IV fluids. Subject responded well to the given treatment. Her blood investigations showed Hb- 10.2, PCV- 30.3, TLC- 4500, platelet- 77000, urea- 35, creatinine- 1.0, GFR- 56.2, sodium- 142, potassium- 3.06, Bilirubin: T- 0.75, D- 0.30, I- 0.45, ALP- 55, SGPT- 13, SGOT- 20, protein- 6.79, albumin- 3.81, globulin- 2.97, ratio- 1.3. 2D Echo showed normal LV dimensions, LA is mildly dilated, borderline LVH, no RWMA of LV at rest, good LV systolic function, LVEF- 63%, global longitudinal strain of LV is 17.1%, LV DD grade-I. During the hospital stay she was treated with antacids, antiemetic's, thyroids supplements, antihypertensive, statins, IV fluids and other supportive medications. Patient responded well to the given treatment, hence is being discharged in haemodynamically stable condition. The event(s) occurred during the treatment period of this study.

No action was taken with respect to study therapy.

At the time of reporting, the event was improving.

The event generalized weakness was considered serious due to hospitalised.

The investigator considered that there was a reasonable possibility of a causal relationship between the study therapy AZD9833/Anastrozole/Placebo and the following event(s): generalized weakness is a symptom of diarrhoea.

The investigator did not consider that there was a reasonable possibility of a causal relationship between the study therapy Palbociclib and the following event(s): generalized weakness is a symptom of diarrhoea.

The company physician did not consider that there was a reasonable possibility of a causal relationship between the study therapy AZD9833/Anastrozole/Placebo and Palbociclib and the event(s): generalized weakness is a symptom of diarrhoea

Summary of insignificant follow-up information received by AstraZeneca/MedImmune 07-May-2024: Narrative updated. Corrected Report 09-May-2024: Suspect placebo added.

Follow-up of insignificant information received by AstraZeneca/MedImmune on 09-May-2024: Narrative Updated.

Summary of follow-up information received by AstraZeneca/MedImmune on 01-Jun-2024: Adverse Event: updated from Generalized Weakness (preferred term: Asthenia)to Generalized Weakness is a symptom of diarrhoea diarrhoea (preferred term: Diarrhoea). Narrative Updated.

	Lab Test/Result Test/Comment	Lab Value	Units	Date	Ref. to Normal	Low	High
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1.	UREA			??-JUL-7000			
	Unknown						
2.	SODIUM						
	Unknown						
3.	SGPT						
	Unknown						
4.	SGOT						
	Unknown						
5.	PROTEIN						
	Unknown						
6.	POTASSIUM						
	Unknown						
7.	INVESTIGATION						
	Unknown						
8.	HB						
	Unknown						
9.	GLOBULIN						

CONTINUED FROM PREVIOUS PAGE	1. PATIENT INITIALS (first, last) UNKNOWN	24b. MFR CONTROL NO. 2024A098631
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Unknown

10. GFR

Unknown

11. CREATININE

Unknown

12. BILIRUBIN

Unknown

13. ALP

Unknown

14. ALBUMIN

Unknown

15. HAEMOGLOBIN

unknown

16. PLATELET

unknown

17. INDICATION(S) FOR USE [Continuation]:

3. BREAST CANCER

18. THERAPY DATE(S) [Continuation]:

05-JAN-2024 to 01-FEB-2024 / 02-FEB-2024 to 01-MAR-2024 / 02-MAR-2024 to 31-MAR-2024 / 01-APR-2024 to UNK

2. 19-MAY-2023 to 08-JUN-2023 / 16-JUN-2023 to 06-JUL-2023 / 14-JUL-2023 to 03-AUG-2023 / 14-AUG-2023 to 03-SEP-2023 / 11-SEP-2023 to 01-OCT-2023 / 09-OCT-2023 to 29-OCT-2023 / 07-NOV-2023 to 06-DEC-2023 / 07-DEC-2023 to 27-DEC-2023 / 05-JAN-2024 to 25-JAN-2024 / 02-FEB-2024 to 22-FEB-2024 / 02-MAR-2024 to 22-MAR-2024 / 01-APR-2024 to UNK

3. 19-MAY-2023 to 15-JUN-2023 / 16-JUN-2023 to 01-APR-2024

19. THERAPY DURATION [Continuation]:

2 days / UNK

2. 3 weeks 0 days / 3 weeks 0 days / 3 weeks 0 days / 3 weeks 0 days / 3 weeks 0 days / 3 weeks 0 days / 4 weeks 2 days / 3 weeks 0 days / 3 weeks 0 days /

3 weeks 0 days / 3 weeks 0 days / UNK

3. 4 weeks 0 days / 41 weeks 4 days

22. CONCOMITANT DRUG(S) AND DATES OF ADMINISTRATION (exclude those used to treat reaction) [Continuation]:

Indications: HYPOTHYROIDISM

3. OROFER XT(FERROUS ASCORBATE, FOLIC ACID): Dates: UNK Duration: UNK Dose: 100 mg DAILY Route: Oral Indications:

3) OROFER XT(FERROUS ASCORBATE + FOLIC ACID)

4. SUPRACAL PRO(BORON, CALCIUM CITRATE, COLECALCIFEROL, COPPER, MAGNESIUM HYDROXIDE, MANGANESE, SILICON, ZINC):

Dates: 16-JUN-2023 to UNK Duration: UNK Dose: 1000 mg DAILY Route: Oral Indications: ANAEMIA

5. ATORVASTATIN(ATORVASTATIN CALCIUM): Dates: 19-MAY-2023 to UNK Duration: UNK Dose: 20 mg DAILY Route: Oral

Indications: CALCIUM SUPPLEMENT

6. (ERYTHROPOIETIN): Dates: 12-MAY-2023 to UNK Duration: 4 days Dose: 1 mL DAILY Route: Subcutaneous Indications:

A HYPERCHOLESTEROLEMIA

7. PANTOPRAZOLE(PANTOPRAZOLE SODIUM SESQUIHYDRATE): Dates: 01-APR-2024 to 04-APR-2024 Duration: 3 days Dose: 40 mg

DAILY Route: Intravenous (not otherwise specified) Indications: ANEMIA

8. (ONDANSETRON): Dates: 23-APR-2024 to 25-APR-2024 Duration: 3 days Dose: 4 mg DAILY Route: Intravenous (not

otherwise specified) Indications: PROPHYLAXIS FOR ACIDITY

9. POTASSIUM CHLORIDE(POTASSIUM CHLORIDE): Dates: 23-APR-2024 to 25-APR-2024 Duration: 3 days Dose: 20 mL Q1H

Route: Intravenous (not otherwise specified) Indications: PROPHYLAXIS FOR NAUSEA AND VOMITING

10. KESOL(POTASSIUM CHLORIDE): Dates: 23-APR-2024 to 25-APR-2024 Duration: 3 days Dose: 15 mL DAILY Route: Oral

Indications: TREATMENT FOR HYPOKALEMIA

23. OTHER RELEVANT HISTORY (e.g. diagnoses, allergies, pregnancy with last menstrual period, etc.) [Continuation]:

Description (MedDRA PT)	Type	Dates
HYSTERECTOMY (Hysterectomy)	History	
PROPHYLAXIS FOR NAUSEA AND VOMITING (Hypokalaemia)	Drug indication	
RIGHT MODIFIED RADICAL MASTECTOMY (Modified radical mastectomy)	History	

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Description (MedDRA PT)	Type	Dates
HYPOTHYROIDISM (Hypothyroidism)	Drug indication	
ANAEMIA (Mineral supplementation)	Drug indication	
HYPERCHOLESTEROLEMIA (Breast cancer)	Current condition	12-MAY-2023-
3) OROFER XT(FERROUS ASCORBATE + FOLIC ACID) (Anaemia)	Drug indication	
A HYPERCHOLESTEROLEMI (Anaemia)	Drug indication	
PROPHYLAXIS FOR ACIDITY (Prophylaxis of nausea and vomiting)	Drug indication	
TREATMENT FOR HYPOKALEMIA (Hypokalaemia)	Drug indication	
HIATUS HERNIA (Hiatus hernia)	History	01-JUN-2021-10-JUL-2021
HYPERTENSION (Hypertension)	Drug indication	
LUMBAR SPINAL SCOLIOSIS (Scoliosis)	History	21-JUL-2021-??-JAN-2022
BILATERAL KNEE REPLACEMENT (Knee arthroplasty)	History	
BREAST CANCER (Breast cancer)	Drug indication	
CALCIUM SUPPLEMENT (Hypercholesterolaemia)	Drug indication	
ANEMIA (Prophylaxis)	Drug indication	